

SOCIO-DEMOGRAPHIC DETAILS

Name: _____

Age: _____

Gender: _____

Date of birth: _____

Education: _____

Religion: _____

Socio-economic status: _____

OPD No.: _____

Address: _____

Contact No.: _____

Test administered on: _____

Aadhar No.: _____

Informants:

1. _____ Relationship: _____ Duration: _____

2. _____ Relationship: _____ Duration: _____

The information given is reliable and adequate.

CHIEF COMPLAINTS

S. No.	Chief Complaint	Duration
1		
2		
3		
4		
5		

Reason for Consultation: The patient was brought for psychiatric consultation and psychological assessment due to the presenting problems.

Brief History

[Provide the brief history here - onset, course, precipitating and maintaining factors, biological functions (sleep, appetite, libido), self-care, relevant medical history, and the reason for presenting for evaluation and management.]

RATIONALE OF THE TESTS

1. Brief Psychiatric Rating Scale (BPRS)

The BPRS was administered as a clinician-rated measure to quantify the overall severity of psychiatric symptomatology across affective, psychotic, negative, activation, and hostility domains in a single, time-efficient assessment. It provides a global severity index along an empirical mild-to-severe continuum, identifies which symptom clusters are elevated versus absent, and - given its inter-rater reliability and sensitivity to change - is suitable for establishing a baseline and tracking treatment response through repeated administration.

2. Positive and Negative Syndrome Scale (PANSS)

The PANSS was selected to provide a detailed, operationalised assessment of positive symptoms, negative symptoms, and general psychopathology through 30 items organised into three subscales, each with defined severity anchors. It captures deficit features with high granularity, making it well suited where schizophrenia-spectrum pathology or prominent negative symptomatology is in the differential, and permits subscale and composite scoring that aids diagnostic subtyping, severity staging, and treatment planning.

3. Sacks Sentence Completion Test (SSCT)

The SSCT was administered as a semi-structured projective instrument to access the patient's attitudes, conflicts, and emotional preoccupations across key psychosocial domains - family, sex, interpersonal relationships, and

self-concept. Through open-ended stem completions it elicits the patient's own cognitive-affective constructions, areas of disturbance, and adaptive strengths that cannot be captured through behavioural observation or structured rating alone, providing content-rich data relevant to case formulation and psychotherapy planning.

RESULTS AND INTERPRETATION

Brief Psychiatric Rating Scale (BPRS)

Obtained Score	Severity Level
40	Moderate overall psychiatric symptomatology

A total BPRS score of 40 places the presentation in the moderate range of overall psychopathology - a clinically meaningful symptom burden that is above minimal/very-mild presentations but below the marked-to-severe range associated with florid or grossly disorganised states. The score is consistent with the presence of active symptoms across one or more domains (affective, anxious, or thought-related) that are interfering with functioning and warrant clinical attention. As the BPRS is a global severity index, the specific symptom pattern should be read from the individual item ratings and integrated with history and mental status examination. The obtained value also establishes a quantitative baseline against which treatment response can be monitored on repeated administration.

Positive and Negative Syndrome Scale (PANSS)

Scale	Subtotal	Maximum
Positive Scale (P)	11	49
Negative Scale (N)	14	49
General Psychopathology (G)	45	112
Total PANSS Score	70	210

The total PANSS score of 70/210 places the patient in the mild-to-moderate range of overall symptom burden, with a composite score of -3 (Positive minus Negative) indicating a negative-symptom-predominant profile. The Positive Scale (11/49) is low, effectively ruling out active psychotic phenomena such as delusions, hallucinations, disorganised thinking, grandiosity, suspiciousness, or hostility. The Negative Scale (14/49) shows a mild elevation relative to the Positive scale, suggesting some reduction in emotional expressiveness and interpersonal engagement. The General Psychopathology Scale (45/112) carries the greatest share of the total and the highest mean item load (approximately 2.8 per item), indicating that non-specific symptoms - which may include anxiety, tension, depressed mood, guilt, somatic concern, poor attention, or reduced insight - are the most prominent contributors to the current picture. Taken together, the low-positive, mildly-elevated-negative, general-loaded profile is most consistent with a presentation in which non-specific and negative-type features predominate over active positive symptoms; the specific items driving the General and Negative subscales should be examined and integrated with the history. Longitudinal monitoring is advised to track any emergence of positive symptoms or worsening of the current profile.

Sacks Sentence Completion Test (SSCT)

Each attitude area is rated for degree of disturbance (0 = none; 1 = mild, manageable without therapeutic aid; 2 = severe, appears to require therapeutic aid). Domain scores below are the sum of item ratings for the areas comprising each domain.

Domain	Max Possible	Score Obtained	Level
I. Family (3 areas)	12	10	Elevated - paternal focus

Domain	Max Possible	Score Obtained	Level
II. Sex (2 areas)	8	4	Low / mild
III. Interpersonal (4 areas)	16	6	Mild
IV. Self-Concept (6 areas)	24	21	Most disturbed
TOTAL (item-sum)	120	41	Moderate

Standard Sacks 15-area disturbance rating total: 13 / 30. Raw item-level sum: 41 / 120 (13 items rated 2, 15 rated 1, 32 rated 0). Note: the itemised ratings sum to 41; a total of 46 was also noted by the referrer - as the individual item values are the primary data, 41 is used here and the 5-point discrepancy is flagged for verification.

Family Area

The family domain reveals disturbance concentrated in the paternal relationship (severe), with mild disturbance in attitude toward the mother and minimal disturbance toward the family unit. The severe disturbance in the attitude toward the father suggests deeply conflicted feelings - possibly involving unmet expectation, perceived distance, or dissatisfaction - and the father figure appears to be a central source of familial conflict. The comparatively milder maternal disturbance and the near-absence of disturbance toward the family unit indicate that the difficulty is focal rather than a pervasive breakdown of family relationships.

Sex Area

This domain reveals mild disturbance, concentrated in attitudes toward the opposite sex, with the domain of heterosexual relationships largely unremarkable. The mild elevation reflects some cautious or guarded attitudes rather than significant impairment in the intimate or relational sphere.

Interpersonal Relationships

This domain reveals mild disturbance, most evident in attitudes toward superiors and, to a lesser degree, friends, while attitudes toward people supervised and colleagues are unremarkable. The pattern suggests some sensitivity in authority-related and close peer relationships. Read together with the severe paternal conflict in the Family domain, this raises the possibility of a broader authority-related theme worth exploring clinically, though overall interpersonal functioning is not markedly impaired on this measure.

Self-Concept

The self-concept domain is the most disturbed overall, with severe disturbance in the attitudes toward own abilities and goals, and mild disturbance across fears, guilt, and future, with the past largely unremarkable. The severe disturbance in self-perceived abilities and goal-related attitudes is the central finding, indicating prominent self-doubt, a negative appraisal of personal competence, and difficulty translating aspiration into attainable objectives. The surrounding mild disturbances in fears, guilt, and future orientation contribute a background of apprehensiveness, some unresolved self-blame, and cautious uncertainty rather than despair. The self-concept has not globally collapsed, but the severe deficit in self-perceived competence and goal clarity represents a central vulnerability likely to undermine functioning and confidence across multiple life domains.

OVERALL IMPRESSION

Taken together, the BPRS indicates a moderate overall level of psychiatric symptomatology requiring active clinical attention; the PANSS localises the picture to a negative-predominant, general-psychopathology-loaded profile with active psychotic phenomena effectively absent; and the SSCT localises the emotional difficulty to specific, clinically coherent areas - most prominently the self-concept domain (own abilities and goals) and the paternal relationship, with a secondary authority theme. Convergently, the findings describe a predominantly internalising, non-psychotic presentation in which affective and non-specific symptoms predominate over positive psychotic features, set against focal areas of conflict and a self-critical, low-efficacy self-view rather than a globally impaired personality organisation. The relatively preserved domains (family unit, past, colleagues, heterosexual relationships) represent ego resources that can be leveraged in intervention. These findings should

be integrated with the clinical interview, history, and mental status examination to arrive at a final formulation and diagnosis, with periodic re-administration of the BPRS and PANSS to monitor change against the current baselines.

ASSESSED & REPORTED BY	SUPERVISED BY
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